

Appendix G Final Exam

1. A medication aide (MA) is not usually allowed to administer a drug
 - (A) If it is the first time a resident takes it
 - (B) If the drug is for a heart condition
 - (C) If the drug is for a chronic illness
 - (D) If a doctor has not provided written approval for the MA to do so
2. Which of the following is an example of drug-disease interaction?
 - (A) A person with hypertension has elevated blood pressure after taking an OTC cold medication.
 - (B) A person with depression has a change in mood after starting a new medication.
 - (C) A person who has been taking a medication for a long time finds that it is less effective.
 - (D) A person who takes medication for hypertension sometimes has a low blood pressure measurement.
3. A nebulizer or inhaler does this to a drug so that it may be inhaled and reach the lungs:
 - (A) Aerosolizes it
 - (B) Dissolves it
 - (C) Concentrates it
 - (D) Injects it
4. Not shaking a liquid medication that requires shaking is an example of
 - (A) Diversion
 - (B) Physical abuse
 - (C) Negligence
 - (D) Fraud
5. Which of these is a topical medication?
 - (A) A powder that is stirred into liquid and swallowed
 - (B) A liquid that is emptied into a machine and made into a mist to be inhaled
 - (C) A fine spray that is administered through the nostrils
 - (D) An ointment that is applied to the skin
6. If a resident's MAR indicates that the pulse rate should be counted before administering a medication, how often does the MA need to count and document the resident's pulse rate?
 - (A) Every time that medication is administered
 - (B) Once a day, when the medication is first administered
 - (C) When the resident looks tired or seems unresponsive
 - (D) When the resident looks anxious or hyperactive
7. Which of the following is an example of respecting a resident's right to participate in her own care?
 - (A) Making sure the resident takes every medication on the MAR no matter what
 - (B) Respecting the resident's right to refuse a medication
 - (C) Giving a resident privacy when she has visitors
 - (D) Promptly reporting any medication errors
8. The therapeutic dose of a medication is high enough to be _____ and low enough to avoid _____.
 - (A) Toxic; effectiveness
 - (B) Effective; toxicity
 - (C) Tolerated; side effects
 - (D) Administered; anaphylaxis
9. Why should healthcare workers not put a zero (0) after a decimal when writing whole numbers?
 - (A) Because it saves time to skip writing the zero
 - (B) Because it takes up less room on residents' charts
 - (C) Because it is an inaccurate way to write numbers
 - (D) Because it could cause a number to be read incorrectly
10. Which of these factors seriously affects the way a drug is used in the body?
 - (A) Whether it is a generic or name-brand drug
 - (B) The age of the person taking the drug
 - (C) The eye color of the person taking the drug
 - (D) How long a person remains seated after taking the drug

11. A type of liquid medication that contains drug particles that are not completely dissolved is a
 - (A) Syrup
 - (B) Elixir
 - (C) Solution
 - (D) Suspension
12. Which of the following medications may be very dangerous if discarded incorrectly?
 - (A) Fentanyl patch
 - (B) Lansoprazole tablets
 - (C) Scopolamine patch
 - (D) Hydrocortisone suppository
13. Which of the following foods is known to cause interactions with many different drugs?
 - (A) Potato chips
 - (B) Chicken
 - (C) Grapefruit
 - (D) Avocado
14. A drug's indication is the _____ it is taken.
 - (A) Route by which
 - (B) Time
 - (C) Reason
 - (D) Number of days
15. What needs to be documented when a PRN medication is administered?
 - (A) The MA's assessment of the resident
 - (B) The reason the medication is being administered and who assessed the resident/authorized administration
 - (C) The length of time since the medication was last administered
 - (D) The date and time of administration only
16. Which of these could be an example of nonverbal communication of medication refusal?
 - (A) The resident sighs when an MA brings in her morning medications.
 - (B) The resident uses a communication board to spell out the word "No."
 - (C) The resident accidentally drops the souffle cup when the MA hands it to her.
 - (D) The resident shakes her head and refuses to take the souffle cup when the MA tries to hand it to her.
17. With whom can an MA share information about a resident's medications?
 - (A) With the resident's son or daughter, whether or not the resident approves
 - (B) With anyone who works in a healthcare setting
 - (C) With the resident and members of the care team
 - (D) With any trusted person
18. Which of the following medications requires careful counting and entry in a special log?
 - (A) Acetaminophen
 - (B) Hydrocodone
 - (C) Lisinopril
 - (D) Warfarin
19. What does it mean for a nurse to *delegate* the task of medication administration?
 - (A) The nurse is trusting the MA to assess when a resident needs a medication.
 - (B) The nurse is transferring responsibility to the medication aide to perform the specific task of administering a drug according to instructions in the MAR.
 - (C) The nurse is allowing the MA to plan administration of a drug with the resident's doctor.
 - (D) The nurse is allowing the resident to tell the MA directly what drugs she needs and when.
20. Which of the following is allowed for an MA?
 - (A) Receiving medication orders by phone
 - (B) Assessing a resident's need for a PRN medication
 - (C) Administering an established oral medication
 - (D) Calculating a medication dose based on a resident's weight
21. Objective information is based on _____ and subjective information is based on _____.
 - (A) Observed facts; reported experiences
 - (B) Reported experiences; observed facts
 - (C) Opinions; data
 - (D) Measurements; research

22. Which of these are always listed in the “rights” of medication administration?
- (A) Right resident, right medication, and right route
 - (B) Right resident, right medication, and right practitioner
 - (C) Right resident, right medication, and right facility
 - (D) Right resident, right time, and right attitude
23. When an MA is administering a medication he has never administered before, what should he do?
- (A) Tell a nurse he cannot administer it because he is not familiar with it
 - (B) Learn more about it using a trusted reference source
 - (C) Ask the resident why the medication was prescribed
 - (D) Ask another MA to tell him about it
24. What is the purpose of reporting all medication errors, even if there is no harm to a resident?
- (A) It allows the person who made the error to be punished appropriately.
 - (B) It alerts the residents and their families that the facility is not safe.
 - (C) It can help improve processes and increase safety.
 - (D) It is city law in all American cities.
25. Which system is the standard system for measurement in health care?
- (A) Household
 - (B) Apothecaries’
 - (C) Standard
 - (D) Metric
26. Which of these describes the best way to measure liquid medications?
- (A) With a dose cup marked in milliliters
 - (B) With a kitchen measuring spoon
 - (C) With a dose cup marked in teaspoons and tablespoons
 - (D) With a household spoon
27. Medication side effects are mostly _____.
- (A) Predictable
 - (B) Dangerous
 - (C) Preventable
 - (D) Imagined
28. An allergic reaction may also be called
- (A) Hypersensitivity
 - (B) Oral candidiasis
 - (C) Paradoxical reaction
 - (D) Hypertension
29. Angioedema is a potentially dangerous side effect of which of these classes of antihypertensive drugs?
- (A) Beta-blockers
 - (B) Potassium-sparing diuretics
 - (C) ACE inhibitors
 - (D) Calcium channel blockers
30. In which case might an MA need to measure and document vital signs?
- (A) Before administering a painkiller such as acetaminophen
 - (B) Before administering a medication that can cause headaches
 - (C) Before administering a medication that affects pulse or blood pressure
 - (D) Before administering a medication for indigestion
31. A resident who experiences excitement and hostility after taking a medication for anxiety may be experiencing
- (A) Hypersensitive reaction
 - (B) Anaphylactic reaction
 - (C) Paradoxical reaction
 - (D) Drug-disease interaction
32. State law may determine the length and type of _____ an MA must have.
- (A) Lanyard
 - (B) Scrubs
 - (C) Medication cart
 - (D) Training

33. A resident with hypothyroidism has been taking iron supplements and levothyroxine at the same time. Her most recent blood tests show that her thyroid levels are too low. Which drug-drug interaction could be the cause of her low thyroid levels?
- (A) Addition
 - (B) Antagonism
 - (C) Potentiation
 - (D) Synergism
34. A(n) _____ medication is administered through the application of a patch to the skin.
- (A) Nebulized
 - (B) Intravenous
 - (C) Buccal
 - (D) Transdermal
35. A resident tells an MA that the tablet she is giving him is the wrong drug. The MA insists that it is correct and that the resident should take it. Later, it is discovered that the pharmacy provided the wrong medication. Which of the following causes of medication errors is involved most directly in this case?
- (A) Failure to follow medication orders exactly
 - (B) Failure to follow instructions for use of a medication
 - (C) Failure to follow a facility's procedures and policies
 - (D) Failure to listen to a resident's or family member's concerns
36. All of these healthcare workers are able to order medications:
- (A) Physician, physician assistant, licensed vocational nurse
 - (B) Physician, clinical pharmacist, dentist
 - (C) Physician, dentist, pharmacy technician
 - (D) Physician, registered nurse, optometrist
37. Which of these reactions must always be observed for when a resident takes an antibiotic?
- (A) Hypertensive emergency
 - (B) Anaphylaxis
 - (C) Headache
 - (D) Lower resistance to infection
38. Residents taking this drug need regular fingerstick blood tests and must eat a consistent amount of vitamin-K-containing foods.
- (A) Clopidogrel (Plavix)
 - (B) Prasugrel (Effient)
 - (C) Apixaban (Eliquis)
 - (D) Warfarin (Coumadin)
39. When should an MA refuse a task?
- (A) When it is something he is not trained to do or that is outside an MA's scope of practice
 - (B) When he sees that the resident does not need the drug he is assigned to administer
 - (C) When he is not sure he can complete all assigned tasks that day
 - (D) When it is assigned by a nurse he does not like
40. When is a boxed warning issued for a medication?
- (A) When it has a particularly bad taste
 - (B) When it is toxic to pets
 - (C) When it is toxic to children
 - (D) When it may cause serious, potentially life-threatening effects
41. Which of the following is an example of a contraindication for a drug?
- (A) Elderly people may be affected more strongly by sedative medications.
 - (B) People with ulcers experience a higher risk of gastrointestinal bleeding if they take NSAIDs.
 - (C) Children need a lower dose of ibuprofen than adults.
 - (D) A resident dislikes swallowing a medication in the form of a large tablet.
42. _____ medications are the most likely to be kept at a facility in traditional packaging.
- (A) Over-the-counter stock
 - (B) Blood pressure
 - (C) Emergency
 - (D) Controlled

43. Which of the following is a requirement to prevent infection during medication administration?
- (A) Medication aides should reuse souffle cups only if they are not visibly soiled.
 - (B) Medication aides should wash hands or use an alcohol-based rub between residents if they did not wear gloves during medication administration.
 - (C) Medication aides should always wash hands or use an alcohol-based rub between residents.
 - (D) Medication aides should ask residents to wear gloves during medication administration.
44. Which of the following is true about glaucoma medications?
- (A) Residents should not take them if they are not experiencing pain or pressure in their eyes.
 - (B) Medication aides will be responsible for deciding whether a resident needs the medications each day.
 - (C) Medications are only necessary in severe cases.
 - (D) Medications are necessary even when there are no symptoms present.
45. What rule applies to administering a medication that has fallen on the ground?
- (A) The “five-second rule” means that it can be administered if it has not been on the ground more than 5 seconds.
 - (B) No medication that has fallen on the ground may be administered.
 - (C) The medication can only be administered if it falls on a carpeted surface.
 - (D) The medication can only be administered if it falls on the floor in the resident’s room rather than in a hallway or other public location.
46. Medications for hypothyroidism must be taken
- (A) Immediately before bedtime
 - (B) Immediately after a meal
 - (C) With a glass of milk
 - (D) On an empty stomach, with no food for an hour afterward
47. Which of the following responses by an MA to resident refusal of a medication is appropriate?
- (A) “Your doctor will be really mad if you don’t take this medication, so please take it.”
 - (B) “If you don’t take this medication, I might get in trouble.”
 - (C) “Can you tell me more about why you don’t want to take this medication?”
 - (D) “Could you please take this medication? Your son will be really disappointed to hear you’re refusing it.”
48. Why must medication aides carefully observe residents with diabetes who take beta-blockers and report dizziness, fainting, loss of balance, and fatigue immediately?
- (A) Because beta-blockers also block insulin
 - (B) Because the symptoms of low blood sugar can be mistaken for side effects of beta-blockers
 - (C) Because high blood pressure can cause diabetic ketoacidosis
 - (D) Because diabetes causes the body to resist the effects of beta-blockers
49. Amoxicillin may be prescribed for which of these conditions of the integumentary system?
- (A) Contact dermatitis
 - (B) Athlete’s foot
 - (C) Psoriasis
 - (D) Cellulitis
50. Hypertensive emergency involves a systolic blood pressure over _____ or a diastolic blood pressure over _____.
- (A) 100, 60
 - (B) 120, 80
 - (C) 140, 90
 - (D) 180, 120
51. Antispasmodic agents, tricyclic antidepressants, and probiotics are all associated with the treatment of which gastrointestinal condition?
- (A) Inflammatory bowel disease
 - (B) Irritable bowel syndrome
 - (C) Diverticulitis
 - (D) Cholelithiasis

52. How does grapefruit change the effects of statin drugs?
- (A) It increases the effects.
 - (B) It decreases the effects.
 - (C) It completely cancels out the effects.
 - (D) It exactly doubles the effects.
53. A(n) _____ may be ordered for a resident with asthma to control symptoms on a regular basis; a _____ may be ordered as “rescue” medications for use during an attack.
- (A) Inhaled corticosteroid; fast-acting bronchodilator
 - (B) Topical corticosteroid; long-acting bronchodilator
 - (C) Oral corticosteroid; leukotriene receptor antagonist
 - (D) Fast-acting bronchodilator; long-acting bronchodilator
54. When should medication administration be documented?
- (A) Immediately after the medication is set up, right before the resident takes it
 - (B) Immediately before the medication is set up, when the MA reaches the resident’s room
 - (C) Immediately after the medication is administered to the resident
 - (D) Immediately before the MA starts a medication pass
55. Medications administered to the eye may be called ophthalmic, optic, or _____ medications.
- (A) Otic
 - (B) Intranasal
 - (C) Parenteral
 - (D) Ocular
56. When are the three safety checks performed before a drug is administered?
- (A) When the drug is prescribed, when it is delivered, and when it is discarded
 - (B) When the drug is removed from storage, when it is taken to the resident’s room, and when it is administered to the resident
 - (C) When the drug is placed in the medication cart, when it is administered to the resident, and when the nurse requests it
 - (D) When the drug is removed from storage, when it is removed from its packaging, and when it is replaced in storage
57. Where should the label on a liquid medication be while the MA pours it into the medication cup?
- (A) Facing the floor
 - (B) Facing the resident
 - (C) Facing the medication aide’s palm
 - (D) Facing the door of the resident’s room
58. What might an MA do after administering oral medications to ensure they have been swallowed?
- (A) Ask the resident if she swallowed them
 - (B) Request that the nurse test the resident’s blood for the drug after an hour
 - (C) Check the resident’s mouth
 - (D) Ask a witness to confirm that they were swallowed
59. Which of the following is an acceptable location for an MA to place a buccal tablet?
- (A) Under the resident’s tongue
 - (B) On the tip of the resident’s tongue
 - (C) Between the gums and the lower lip
 - (D) On the roof of the resident’s mouth
60. ADME stands for
- (A) Administration, Development, Malabsorption, and Elimination
 - (B) Absorption, Distribution, Metabolism, and Excretion
 - (C) Addition, Division, Multiplication, and Extraction
 - (D) Administration, Distribution, Medication, and Evolution

61. DMARDs are drugs used in the treatment of
(A) Cancer
(B) HIV/AIDS
(C) Rheumatoid arthritis
(D) Shingles
62. How does an MA indicate on the MAR that a medication has been administered?
(A) By placing an X on the MAR
(B) By initialing the MAR
(C) By placing a circle on the MAR
(D) By writing the time of administration on the MAR
63. Antiviral medications for influenza are usually most effective when taken
(A) Before a person is exposed to influenza
(B) After the most severe symptoms develop
(C) As early as possible in the course of the illness
(D) Throughout all of flu season
64. Where are ophthalmic medications administered?
(A) Straight into the eye
(B) Straight into the ear
(C) Into the inside of the lower eyelid
(D) Into the outer edge of the eye
65. Which of the following side effects is a *systemic* side effect?
(A) Injection site pain after a flu shot
(B) Rash after administration of an ointment
(C) Oral thrush after use of an inhaled corticosteroid
(D) Hypertension resulting from use of an oral corticosteroid
66. Residents with gout whose symptoms are not controlled by diet may be prescribed a medication that reduces _____ in the body.
(A) Digestive enzymes
(B) Uric acid
(C) Platelet count
(D) Calcium levels
67. What is the reason for pulling back and up on the outside edge of the ear when administering otic medications?
(A) This straightens the ear canal.
(B) This serves as a warning to the resident that the drops are about to be administered.
(C) This allows drops to fall straight down into the ear without touching the sides of the ear canal.
(D) This prevents the drops from creating dizziness when they are administered.
68. Residents taking immunosuppressants must be observed closely for
(A) Signs of infection
(B) Signs of depression
(C) Changes in level of consciousness
(D) Signs of bleeding
69. A controlled release medication usually has a _____ that affects how it is processed in the body.
(A) Coating
(B) Scoring
(C) Color
(D) Shape
70. Residents who take diuretics must be monitored to ensure that _____ and _____ are maintained at healthy levels.
(A) Fluid balance; potassium
(B) Intestinal microbes; iron
(C) Iron; white blood cells
(D) Vitamin B12; fluid balance
71. Which of the following is true about transdermal medications?
(A) The old patch should be left on when a new patch is placed.
(B) The old patch should be removed before a new patch is placed.
(C) It does not matter if an old patch is removed or not because it no longer contains any medicine.
(D) The old patch is only removed if it is an opioid medication.

72. If a medication classified as a controlled substance must be destroyed or disposed of, this task generally must be performed
- (A) By the doctor who prescribed it
 - (B) By the charge nurse
 - (C) By a facility's administrator
 - (D) With two staff members present
73. A nurse determines that a resident should receive acetaminophen for a headache, as indicated on her MAR. The MA instead administers aspirin. Which of the following is true?
- (A) This medication error is definitely not significant and does not need to be reported.
 - (B) This medication error is definitely not significant but does need to be reported.
 - (C) This medication error is definitely significant and needs to be reported.
 - (D) This medication error may be significant, depending on the resident, and needs to be reported.
74. Mrs. Okonta has a PRN order for a nebulized medication. A nurse assesses Mrs. Okonta and assigns Janet, an MA, to administer the nebulized medication. State law and facility policy allow MAs to administer this type of drug. Which of the following is true?
- (A) Janet can administer this medication only to Mrs. Okonta and only when directed by the nurse.
 - (B) Janet can now administer this medication to any resident at any time.
 - (C) Janet can now administer this medication to Mrs. Okonta any time.
 - (D) Janet can administer this medication to Mrs. Okonta as many times as she needs it over the course of 1 week.
75. A resident experiencing insulin reaction needs to receive _____ immediately.
- (A) Glucose
 - (B) Insulin
 - (C) Metformin or another antidiabetic agent
 - (D) An epinephrine injection
76. What follow-up documentation may be required after a PRN medication is administered?
- (A) The number of times the resident has requested the drug previously
 - (B) The resident's response to the medication
 - (C) Whether the MA thinks the resident should take another dose later in the day
 - (D) The resident's pulse and blood pressure measurements
77. Residents who take antimotility agents must be observed to ensure they do not experience _____ as a result.
- (A) Fever
 - (B) Sweating
 - (C) Constipation
 - (D) Sedation
78. What should an MA do if a resident says his medication does not look like the correct one and that he does not want to take it?
- (A) Insist that the resident take it
 - (B) Show the resident the label and then tell him he needs to take the medication
 - (C) Slip the medication into applesauce to make sure the resident takes it
 - (D) Inform the nurse that the resident is unsure about a medication
79. When should a medication error be reported?
- (A) As soon as it occurs
 - (B) As soon as the MA can determine what effects it may have had
 - (C) As soon as the MA completes the medication pass for all residents
 - (D) At the end of the MA's shift
80. Which of the following is true of medications that are past their expiration date?
- (A) They should only be administered with a doctor's approval.
 - (B) They should only be administered if the expiration date was within the past week.
 - (C) They should not be administered.
 - (D) They should only be administered after the MA checks that they do not have an unusual smell or appearance.

81. Psoriasis and eczema may both be treated with
(A) Topical corticosteroids
(B) Miticides
(C) Oral antifungals
(D) Topical antibiotics
82. Which of these drug classes is associated with increased risk of bleeding in the digestive tract?
(A) Antibiotics
(B) Antifungals
(C) NSAIDs
(D) Anxiolytics
83. Bisphosphonates, ordered to treat osteoporosis, must be administered
(A) Right before a resident goes to bed
(B) On a full stomach, preferably right after breakfast
(C) First thing in the morning, before any other medicine, food, or drinks
(D) Along with a drug that will coat the esophagus and stomach to prevent irritation
84. Which of these medications may be ordered for a resident who has had a stroke or who is considered to be at high risk for a stroke?
(A) Clopidogrel (Plavix)
(B) Lorazepam (Ativan)
(C) Carbidopa/levodopa (Sinemet)
(D) Rivastigmine (Exelon)
85. HIV/AIDS is treated with a combination of different drugs. This is known as
(A) Antiretroviral therapy
(B) Disease modulatory therapy
(C) Messenger RNA therapy
(D) Radiation therapy
86. Which of these sets of drug classes may be ordered for a resident with coronary artery disease?
(A) Diuretics, antipyretics, anxiolytics
(B) Analgesics, antibiotics, nitrates
(C) Antiarrhythmic agents, beta-blockers, and immunosuppressants
(D) Statins, ACE inhibitors, antiplatelet agents
87. What should an MA ask a resident to do before administering an intranasal medication?
(A) Use the restroom
(B) Blow his nose
(C) Cough to clear any mucus in the lungs
(D) Hold his breath
88. Which of the following respiratory illnesses usually requires treatment with antibiotics?
(A) Asthma
(B) Bronchitis
(C) Pneumonia
(D) Influenza
89. _____ is a common symptom of urinary tract infection in elderly people.
(A) Shortness of breath
(B) Headache
(C) Body aches
(D) Confusion
90. Which of the following is true about medications for kidney failure?
(A) Residents with kidney failure are treated exclusively with dialysis; no medications are required.
(B) Residents with kidney failure will likely take beta-blockers, cholinergic agents, and urinary antispasmodic agents.
(C) Residents with kidney failure are unable to metabolize medications and so they will not take any.
(D) Residents with kidney failure may take antihypertensive drugs and statins.
91. These two classes of drugs are both commonly used to treat gastroesophageal reflux disease (GERD):
(A) Proton pump inhibitors and H2 blockers
(B) H2 blockers and antibiotics
(C) Antiemetics and proton pump inhibitors
(D) Antimotility agents and antiemetics

92. A resident's MAR indicates that blood pressure should be checked and documented before administration of a blood pressure medication. It also indicates the medication should be held if the resident's systolic blood pressure is below 120. An MA forgets to check blood pressure and administers the drug. The resident's blood pressure drops dangerously low afterwards and the resident loses consciousness. Which of the following causes of medication errors is involved most directly in this case?
- (A) Failure to follow medication orders exactly
 - (B) Failure to follow instructions for use of a medication
 - (C) Failure to follow a facility's procedures and policies
 - (D) Failure to listen to a resident's or family member's concerns
93. If medications are administered at a facility using a medication cart, which of the following is an important safety measure?
- (A) The MA should prepare medications for the next resident in the room of the resident who just received medications before wheeling the cart out.
 - (B) The MA should prepare medications for every resident and have them organized on the top of the cart before doing a medication pass.
 - (C) The MA should keep the cart locked any time it is unattended.
 - (D) The MA should wheel the cart back to the medication room between residents to prepare the next resident's medications.
94. Which of the following is true of the medication aide's role in cancer treatment?
- (A) MAs will not encounter residents undergoing cancer treatment.
 - (B) MAs will be specially trained to administer IV chemotherapy drugs.
 - (C) MAs are prohibited from administering any kind of medication to residents undergoing cancer treatment.
 - (D) MAs may administer drugs to help residents with cancer manage the side effects of chemotherapy drugs.
95. How should an MA remove a tablet from a blister pack to prepare it for administration?
- (A) He should push it through the foil backing into the palm of his hand, and then place it in a souffle cup.
 - (B) He should push it through the foil backing directly into the resident's mouth.
 - (C) He should cut the "blister" containing the tablet out of the card with scissors, then place the sealed tablet in a souffle cup for the resident to open.
 - (D) He should press it through the foil backing and into a souffle cup.
96. A facility is short-staffed and an MA is asked by a nurse to administer a subcutaneous insulin injection. The syringe has the correct amount of insulin predrawn, but facility policy does not allow MAs to administer medications by any injection route. What should the MA do?
- (A) Tell the nurse they cannot perform the injection
 - (B) Perform the injection if they have been trained to do so
 - (C) Look up a procedure for subcutaneous injection and then perform the injection
 - (D) Ask another MA to assist with the injection
97. Which of the following can shorten the length and reduce the severity of a shingles outbreak?
- (A) NSAIDs
 - (B) Antiviral drugs
 - (C) Antibiotics
 - (D) Tricyclic antidepressants
98. How should the resident's head be positioned during the administration of a nasal spray?
- (A) Slightly forward
 - (B) Slightly back
 - (C) Slightly to the side of the nostril receiving the spray
 - (D) Slightly to the side of the nostril not receiving the spray

99. Metformin (Glucophage) and glipizide (Glucotrol) are medications typically prescribed for this condition:
- (A) Type 1 diabetes
 - (B) Type 2 diabetes
 - (C) Hypothyroidism
 - (D) Hyperthyroidism
100. Which of the following is true about anaphylaxis?
- (A) It usually passes quickly and without harm.
 - (B) It can be life-threatening and requires immediate action.
 - (C) It can only happen in people who have had previous allergic reactions.
 - (D) If a resident has taken a medication before and did not experience anaphylaxis, that medication will never cause anaphylaxis for the resident.